

The colonial roots of our collections and our response

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Wellcome Collection is built on collections assembled by Sir Henry Wellcome. In the early 20th century Wellcome founded a private historical medical museum closely linked with his pharmaceutical business. His staff collected vast quantities of objects, images, archives and books from all over the world, to achieve a better understanding of what he thought of as the "art and science of healing throughout the ages".

His museum, in common with others of the time, followed a late-19th-century European model of cultural hierarchies. Objects were classified and displayed in a way that placed European culture at the top of a racist, sexist and ableist system of cultural dominance.

Between 1890 and 1936 Wellcome built a collection that told a global story of health and medicine in which Black people, Indigenous peoples and people of colour were exoticised, marginalised and exploited.

Wellcome's collection grew through mass acquisitions from the British auction and dealer market. His museum purchased entire collections from other collectors, and used a network of agents in Europe, the Middle East, the Americas and South Asia. By the time he died in 1936, his collection was one of the largest in Europe.

Colonialism and context

We have often described Sir Henry Wellcome as a philanthropist. During his lifetime he supported philanthropic causes, and on his death his will provided for the establishment of the Wellcome Trust, a charitable foundation charged with improving human health.

Such philanthropy was only possible because of Wellcome's accumulation of wealth through his pharmaceutical business. This business benefited greatly from Indigenous medical knowledge, and from imperial policies and trading relationships that subjugated and exploited Indigenous peoples.

A large proportion of the items collected by Henry Wellcome came from non-European cultures. Some were acquired directly from the people who made and owned them, but the majority were bought at auction, often with little information about how they got there.

Following Wellcome's death, the Wellcome Trust dispersed 90 per cent of the objects in his collection to other museums, considering them less relevant to a Eurocentric history of medicine. We have continued to collect books, manuscripts and paintings, and have used these collections to present stories that privileged European medicine and the achievements of individual European scientists.

Nevertheless, our museum and library collections, some of which are now jointly held with the Science Museum, still include many items that were unjustly taken from the people and communities who made them. The agents and collectors who did this were able to do so because colonial structures of violence and control allowed them to.

Items in our collections have been separated from their original context. They have been used to tell a colonial version of the history of health and medicine that privileges European medical understanding over Indigenous and other forms of local knowledge. This version of history uses racist theories and language to present a racist, sexist and ableist story of medicine.

With this legacy constantly in mind, we continue to build the collections today, trying to find different ways to acknowledge the lived experiences of those who continue to be silenced, erased and ignored.

Changing how we look at our collections

We have begun a series of initiatives to change the ways in which we manage and use our collections:

- We are conducting a major inventory of our collections, with the aim of providing equitable access to all collections and surfacing previously marginalised content.
- We have digitised and made available archival records that document the acquisition and dispersal of our historic collections, and we plan to make the information they contain more transparent and accessible.

- We are supporting a diverse research community that can engage critically with our collections, fostering knowledge about them that does not exclusively belong to us.

We want to connect the objects, images and archives in our collections with people for whom they hold significance. We want to engage with voices that will challenge our assumptions, breathe new life and meaning into our collections, and change our understanding of the objects in our care.

This will be a sustained and open-ended process. It will involve collaborating on research into where our objects came from and being transparent about what we learn. Where the history of an object is lost or has been erased, we will connect it with people whose life or health experiences can help us to understand the meanings it might hold today.

We will consult respectfully and proceed appropriately in relation to human remains and sacred objects in our care. We welcome [claims for the restitution](#) of such artefacts and will continue to enable restitution where appropriate.

We are learning from museums and communities where strategies for this process of consultation, reconciliation and restitution are more advanced than they are in Europe, thanks to closer relationships with communities of origin.

Injustice and transparency

We need to confront the uncomfortable history of our collections. We have a responsibility to be honest and transparent about the past injustices in which our collections are rooted.

We are working with visitors, research partners, communities, peer institutions, artists and activists to help us confront and respond to these injustices. We commit to listening to our visitors and to our research partners, to sharing the results of our research, and to being clear about the changes we are making. With their help, we will engage publicly with past injustices to prevent future injustice.